GOVCARE EHR

CKD AI CLINICAL DECISION SUPPORT REPORT

Clinical decision support only - clinician review required

PATIENT & REPORT DETAILS

Patient	Operational software QA; not a clinical subject
Patient number	OPERATIONAL-FEATURE-QA
DOB	Not recorded
Sex	Not recorded
Assessment ID	4cc59f36-127c-43a2-9e76-3335d31633c7
Assessment time	Not recorded
Hospital	Not recorded
Department	Not recorded
Clinician / operator	Not recorded
Model	Software verification only

CKD RISK SUMMARY

Predicted CKD risk	Risk category	AI confidence
N/A	**N/A**	**N/A**

CKD CLASSIFICATION & CLINICAL CODING

Suggested CKD classification	Unable to classify
G category	Not assigned
Suggested ICD code	Not assigned
Clinical description	Not assigned
Terminology system	Not assigned
Basis	Reviewed eGFR unavailable
Coding status	AI-assisted suggestion
Confirmation	Clinician confirmation required

A single eGFR result does not independently establish chronic kidney disease or chronicity. N18.6 (ESRD) is not inferred from eGFR alone.

○ G1	○ G2	○ G3a	○ G3b	○ G4	○ G5

RENAL & ASSOCIATED FINDINGS

Parameter	Result	Unit	Source
eGFR	35.0	mL/min/1.73m²	MANUAL
Serum creatinine	2.0	mg/dL	MANUAL
Urine albumin/creatinine ratio (ACR)	420.0	mg/g	MANUAL
Urine protein	2.0	mg/dL	MANUAL
Systolic blood pressure	148.0	mmHg	MANUAL

Diastolic blood pressure	92.0	mmHg	MANUAL
HbA1c	7.1	%	MANUAL
BMI	28.0	kg/m²	MANUAL
Serum potassium	5.8	mmol/L	MANUAL
Hemoglobin	9.8	g/dL	MANUAL

CKD-RELATED CLINICAL RISKS & PREDICTIVE INTELLIGENCE

Sri Lankan Longitudinal Renal & Hemodialysis Data

Saved snapshot: c9908fe1-85cc-4a21-9498-a13b410d7b3a | Evaluated 2026-10-12T09:44:36.507320623Z. Export reads saved clinical evidence; no inference is performed.

CKD Progression Risk

CALCULATED | High CKD progression monitoring signal

Clinical Data Readiness: 100% Ready | 9/9 parameters | Missing: 0

Monitoring level: High

Qualitative measurement-based monitoring band; no future-event probability or forecast horizon is estimated.

Why: Reduced eGFR: 35.00 mL/min/1.73 m2.

Why: Elevated ACR: 420.00 mg/g.

Why: Serum creatinine: 2.00 mg/dL; interpret against the patient's source reference range.

Measured trend: 1.6 to 2 mg/dL | 2026-09-11T09:16:37.590Z to 2026-10-11T09:16:37.590Z

Previous dated observations available; trends are shown separately from snapshot risk.

Measurement-based monitoring; no new disease probability or confirmed diagnosis. Source details, rules and clinical recommendations follow.

Kidney Failure Risk Indicator

CALCULATED | Kidney failure risk indicator calculated

Clinical Data Readiness: 80% Ready | 4/5 parameters | Missing: 1

Monitoring level: Below high-signal threshold

High monitoring signal requires eGFR <15 or verified chronic dialysis; creatinine alone cannot establish G5D.

Why: eGFR 35.00 mL/min/1.73 m2 does not meet the <15 high-signal threshold.

Dialysis verification required

Needs: Verified dialysis status

Measurement-based monitoring; no new disease probability or confirmed diagnosis. Source details, rules and clinical recommendations follow.

Hyperkalemia Monitoring

CALCULATED | High potassium monitoring signal

Clinical Data Readiness: 100% Ready | 1/1 parameters | Missing: 0

Measured: 5.8 mmol/L | 2026-10-11T09:16:37.590Z

Monitoring level: High

Current measurement monitoring is independent of the maintenance-HD research forecast. The low band does not exclude hypokalemia or other abnormalities.

Why: Serum potassium 5.80 mmol/L; configured bands: <5.0, 5.0-5.5 inclusive, >5.5.

Measured trend: 5 to 5.8 mmol/L | 2026-09-11T09:16:37.590Z to 2026-10-11T09:16:37.590Z

Separate HD research forecast: Eligible dialysis context not verified

Measurement-based monitoring; no new disease probability or confirmed diagnosis. Source details, rules and clinical recommendations follow.

Anemia / Hb Deterioration

CALCULATED | Anemia monitoring signal

Clinical Data Readiness: 100% Ready | 1/1 parameters | Missing: 0

Measured: 9.8 g/dL | 2026-10-11T09:16:37.590Z

Monitoring level: High

Measured Hb monitoring and decline rate are separate from the installed persistence baseline; the baseline is not an anemia prognosis.

Why: Hemoglobin 9.80 g/dL; configured bands: <10, 10-12 inclusive, >12. These are monitoring bands, not universal normal ranges or an anemia diagnosis.

Why: Hb change -1.50 g/dL over 30.00 days; decline rate is positive for a fall and negative for a rise.

Measured trend: 11.3 to 9.8 g/dL | 2026-09-11T09:16:37.590Z to 2026-10-11T09:16:37.590Z

Hb decline rate: 0.05 g/dL/day

Separate HD research forecast: Eligible dialysis context not verified

Measurement-based monitoring; no new disease probability or confirmed diagnosis. Source details, rules and clinical recommendations follow.

Dialysis Adequacy / URR

INSUFFICIENT DATA | Dialysis session measurements required

Clinical Data Readiness: 0% Ready | 0/3 parameters | Missing: 3

Requires hemodialysis session data

Needs: Pre-HD Urea

Needs: Post-HD Urea

Needs: Dialysis date

Separate HD research forecast: Eligible dialysis context not verified

Measurement-based monitoring; no new disease probability or confirmed diagnosis. Source details, rules and clinical recommendations follow.

AKI Risk / Creatinine Trend

CALCULATED | Creatinine change calculated

Clinical Data Readiness: 100% Ready | 4/4 parameters | Missing: 0

Monitoring level: Trend only

Creatinine change is a measured trend. It does not establish AKI; timing, urine output and clinical context require review.

Why: Creatinine 1.60 to 2.00 mg/dL over 30.00 days; 25.00% change (not probability).

Measured trend: 1.6 to 2 mg/dL | 2026-09-11T09:16:37.590Z to 2026-10-11T09:16:37.590Z

Measured creatinine change: 25% (not probability)

Measurement-based monitoring; no new disease probability or confirmed diagnosis. Source details, rules and clinical recommendations follow.

Clinical Decision Support / Research Use. Outputs require clinician interpretation. Research-model outputs are not independently validated diagnoses.

Prediction generated from available clinical measurements. Longitudinal forecasting requires repeated dated observations. Clinical data readiness is input completeness, not diagnostic certainty or disease probability.

SIX-MODULE EVIDENCE & CLINICIAN REVIEW

CKD Progression / Renal Trajectory

CALCULATED - High CKD progression monitoring signal

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Transparent snapshot monitoring rule

GovCare qualitative eGFR/ACR grid: High if eGFR <45, ACR >300, or eGFR <60 with ACR ≥30; Moderate if eGFR <60 or ACR ≥30; Low requires eGFR ≥60 AND ACR <30. Categories condense high/very-high monitoring; not a validated progression model.
Serum Creatinine: 2 mg/dL | 2026-10-11T09:16:37.590Z | Source: CKD_ASSESSMENT / 4cc59f36-127c-43a2-9e76-3335d31633c7
eGFR: 35 mL/min/1.73m² | 2026-10-11T09:16:37.590Z | Source: CKD_ASSESSMENT / 4cc59f36-127c-43a2-9e76-3335d31633c7
Protein in urine: 2 mg/dL | 2026-10-11T09:16:37.590Z | Source: CKD_ASSESSMENT / 4cc59f36-127c-43a2-9e76-3335d31633c7
Urine ACR: 420 mg/g | 2026-10-11T09:16:37.590Z | Source: CKD_ASSESSMENT / 4cc59f36-127c-43a2-9e76-3335d31633c7
Age: 56 years | 2026-10-11T09:16:37.590Z | Source: CKD_ASSESSMENT / 4cc59f36-127c-43a2-9e76-3335d31633c7
Systolic Blood Pressure: 148 mmHg | 2026-10-11T09:16:37.590Z | Source: CKD_ASSESSMENT / 4cc59f36-127c-43a2-9e76-3335d31633c7
Diastolic Blood Pressure: 92 mmHg | 2026-10-11T09:16:37.590Z | Source: CKD_ASSESSMENT / 4cc59f36-127c-43a2-9e76-3335d31633c7
Fasting Blood Sugar: 140 mg/dL | 2026-10-11T09:16:37.590Z | Source: CKD_ASSESSMENT / 4cc59f36-127c-43a2-9e76-3335d31633c7
HbA1c: 7.1 % | 2026-10-11T09:16:37.590Z | Source: CKD_ASSESSMENT / 4cc59f36-127c-43a2-9e76-3335d31633c7

Clinical recommendations

Clinician review of eGFR, ACR, urine findings and renal chronicity; obtain previous dated observations for longitudinal forecasting.
Why: Reduced eGFR: 35.00 mL/min/1.73 m2.
Why: Elevated ACR: 420.00 mg/g.
Why: Serum creatinine: 2.00 mg/dL; interpret against the patient's source reference range.
Why: HbA1c / glucose are supporting glycemic measurements; a diabetes diagnosis is not inferred.
Why: Serum Creatinine increased: 1.60 to 2.00 mg/dL across the recorded source dates.
Why: eGFR decreased: 45.00 to 35.00 mL/min/1.73m2 across the recorded source dates.
Why: Urine ACR increased: 180.00 to 420.00 mg/g across the recorded source dates.
Available: Serum Creatinine | 1/1 | 2.00 mg/dL; source date recorded
Available: eGFR | 1/1 | 35.00 mL/min/1.73m²; source date recorded
Available: Protein in urine | 1/1 | 2.00 mg/dL; source date recorded
Available: Urine ACR | 1/1 | 420.00 mg/g; source date recorded
Available: Age | 1/1 | 56.00 years; source date recorded
Available: Systolic Blood Pressure | 1/1 | 148.00 mmHg; source date recorded
Available: Diastolic Blood Pressure | 1/1 | 92.00 mmHg; source date recorded
Available: Fasting Blood Sugar | 1/1 | 140.00 mg/dL; source date recorded
Available: HbA1c | 1/1 | 7.10 %; source date recorded

Next action: Add historical laboratory data

Directions compare the two latest valid values per domain using a 1% neutral band; this is not a prognosis or a G1-to-G5 forecast.
Dialysis timing, clinical context and measurement uncertainty require clinician interpretation.
Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Kidney Failure / G5D

CALCULATED - Kidney failure risk indicator calculated

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Clinical evidence / monitoring rule

eGFR: 35 mL/min/1.73m² | 2026-10-11T09:16:37.590Z | Source: CKD_ASSESSMENT / 4cc59f36-127c-43a2-9e76-3335d31633c7
Serum Creatinine: 2 mg/dL | 2026-10-11T09:16:37.590Z | Source: CKD_ASSESSMENT / 4cc59f36-127c-43a2-9e76-3335d31633c7
Protein in urine: 2 mg/dL | 2026-10-11T09:16:37.590Z | Source: CKD_ASSESSMENT / 4cc59f36-127c-43a2-9e76-3335d31633c7
Urine ACR: 420 mg/g | 2026-10-11T09:16:37.590Z | Source: CKD_ASSESSMENT / 4cc59f36-127c-43a2-9e76-3335d31633c7

Clinical recommendations

Review renal function and chronicity; verify dialysis status before G5D labeling or maintenance-HD model use.
Why: eGFR 35.00 mL/min/1.73 m2 does not meet the <15 high-signal threshold.
Available: eGFR | 1/1 | 35.00 mL/min/1.73m²; source date recorded
Available: Serum Creatinine | 1/1 | 2.00 mg/dL; source date recorded
Available: Protein in urine | 1/1 | 2.00 mg/dL; source date recorded
Available: Urine ACR | 1/1 | 420.00 mg/g; source date recorded
Needs review / data: Verified dialysis status | 0/1 | Dialysis verification required

Next action: Verify dialysis status

High creatinine alone does not establish G5D. This card reports recorded clinical evidence, not a diagnosis generated by AI.
No 2-year or 5-year kidney-failure probability is calculated.
Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Hyperkalemia Monitoring

CALCULATED - High potassium monitoring signal

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Current potassium monitoring rule

Serum Potassium: 5.8 mmol/L | 2026-10-11T09:16:37.590Z | Source: CKD_ASSESSMENT / 4cc59f36-127c-43a2-9e76-3335d31633c7

Clinical recommendations

Review measured potassium with the clinician, source quality, medications and sampling context; use true paired pre/post-HD samples when available.

Why: Serum potassium 5.80 mmol/L; configured bands: <5.0, 5.0-5.5 inclusive, >5.5.
Available: Serum Potassium | 1/1 | 5.80 mmol/L; source date recorded
Next action: Add historical potassium result
Research outputs require clinician interpretation; internal evaluation is not independent clinical validation.
Dataset permission status remains UNVERIFIED.
This installed model is scoped to verified maintenance hemodialysis. No applicable dialysis context is recorded.
Requires two fresh verified HD observations 14–62 days apart, a current target result and five supported predictor values.
Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Anemia / Hb Deterioration

CALCULATED - Anemia monitoring signal

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Current hemoglobin monitoring rule
Hemoglobin: 9.8 g/dL | 2026-10-11T09:16:37.590Z | Source: CKD_ASSESSMENT / 4cc59f36-127c-43a2-9e76-3335d31633c7

Clinical recommendations

Clinician review against age/sex-specific laboratory ranges, iron studies and clinical context; add a previous dated Hb result for deterioration monitoring.
Why: Hemoglobin 9.80 g/dL; configured bands: <10, 10-12 inclusive, >12. These are monitoring bands, not universal normal ranges or an anemia diagnosis.
Why: Hb change -1.50 g/dL over 30.00 days; decline rate is positive for a fall and negative for a rise.
Available: Hemoglobin | 1/1 | 9.80 g/dL; source date recorded
Next action: Add historical Hb result
Research outputs require clinician interpretation; internal evaluation is not independent clinical validation.
Dataset permission status remains UNVERIFIED.
Learned candidates failed the gate against persistence. Carries forward the last measured Hb; no anemia-classifier probability is installed.
This installed model is scoped to verified maintenance hemodialysis. No applicable dialysis context is recorded.
Requires two fresh verified HD observations 14–62 days apart, a current target result and five supported predictor values.
Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Dialysis Adequacy / URR

INSUFFICIENT DATA - Dialysis session measurements required

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Measured same-session urea reduction ratio
URR = ((Pre-HD Urea − Post-HD Urea) / Pre-HD Urea) × 100

Clinical recommendations

Review measured adequacy with the clinician; record actual pre-HD urea, post-HD urea and dialysis date. BUN from the CKD form is not a paired dialysis urea measurement.
Needs review / data: Pre-HD Urea | 0/1 | Requires hemodialysis session data
Needs review / data: Post-HD Urea | 0/1 | Requires hemodialysis session data
Needs review / data: Dialysis date | 0/1 | Actual dialysis session date; upload time is not substituted
Next action: Add dialysis-session data
Research outputs require clinician interpretation; internal evaluation is not independent clinical validation.
Dataset permission status remains UNVERIFIED.
This installed model is scoped to verified maintenance hemodialysis. No applicable dialysis context is recorded.
Requires two fresh verified HD observations 14–62 days apart, a current target result and five supported predictor values.
Clinical feature sources: 0 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

AKI / Renal Clinical Deterioration

CALCULATED - Creatinine change calculated

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Dated creatinine change / acute review rule
Creatinine-based clinical review only: rise ≥0.3 mg/dL within 48 h or ≥1.5 times a dated baseline within 7 days. No AKI staging, urine-output assessment or diagnosis is generated.
Creatinine change (%) = ((Current Creatinine − Previous Creatinine) / Previous Creatinine) × 100
Serum Creatinine: 2 mg/dL | 2026-10-11T09:16:37.590Z | Source: CKD_ASSESSMENT / 4cc59f36-127c-43a2-9e76-3335d31633c7

Clinical recommendations

Obtain previous creatinine with its actual measurement date; clinician review of timing, acute illness, medications and urine output is required.
Why: Creatinine 1.60 to 2.00 mg/dL over 30.00 days; 25.00% change (not probability).
Available: Serum Creatinine | 1/1 | 2.00 mg/dL; source date recorded
Available: Previous creatinine | 1/1 | Baseline creatinine required for AKI trend calculation
Available: Current creatinine measurement date | 1/1 | Current and previous results need distinct actual dates
Available: Previous creatinine measurement date | 1/1 | A recorded baseline date is required
Next action: Add baseline creatinine history
Directional composite – not a validated prognosis score.
Equal weights; two latest valid observations per domain; 1% neutral stability band. Missing domains are excluded, never scored as zero.
A directional percentage is not a disease probability.
Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

CLINICAL DATA USED FOR ASSESSMENT

Parameter	Result	Unit	Source	Coding
Serum creatinine	2.0	mg/dL	MANUAL	-
eGFR	35.0	mL/min/1.73m²	MANUAL	-
Urine albumin/creatinine ratio (ACR)	420.0	mg/g	MANUAL	-
Serum potassium	5.8	mmol/L	MANUAL	-
Hemoglobin	9.8	g/dL	MANUAL	-
HbA1c	7.1	%	MANUAL	-
Urine protein	2.0	mg/dL	MANUAL	-
Age	56.0	years	MANUAL	-
BMI	28.0	kg/m²	MANUAL	-
Systolic blood pressure	148.0	mmHg	MANUAL	-
Diastolic blood pressure	92.0	mmHg	MANUAL	-
Fasting blood sugar	140.0	mg/dL	MANUAL	-
Blood urea nitrogen (BUN)	30.0	mg/dL	MANUAL	-
Edema	0.0	boolean	MANUAL	-
Family history of kidney disease	0.0	boolean	MANUAL	-
Previous acute kidney injury	0.0	boolean	MANUAL	-
NSAID use	3.0	assessment scale	MANUAL	-

KEY FACTORS IN THIS MODEL OUTPUT

Contribution direction describes this model's prediction, not whether a clinical value is healthy, safe or protective.

Factor	Value	Unit	Direction	Contribution

RECOMMENDED CLINICAL REVIEW

No recommendation was returned by the prediction service.

CLINICIAN REVIEW

Suggested classification	Not assigned
Suggested code	Not assigned
Coding decision	[ ] Confirm [ ] Modify [ ] Reject
Final clinician-selected code	_____
Clinical notes	_____ _____
Reviewed by / date	_____

This output does not diagnose disease or replace professional medical judgement. AI-assisted coding must be reviewed before a confirmed FHIR Condition is created.